250242503838860

Ms. SHASHIKALA N
NO 346, 8TH A MAIN ROAD,
RAJAJINAGAR ,,Bangalore North Bangalo..
Tel No : +919886259840
PIN No: 560055
PID NO: P1832100080128
Age: 50.5 Year(s) Sex: Female

Reference: **DR.MAHESH CHANDRA PAI**
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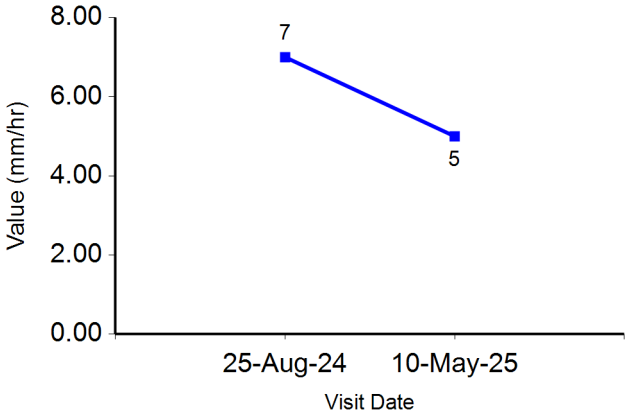
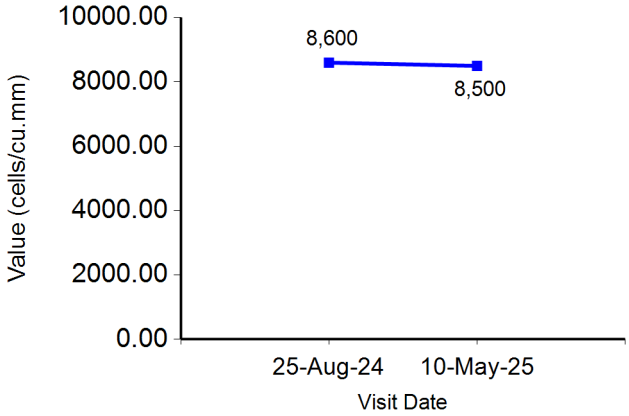
Result Trend (For selected tests used for followup)

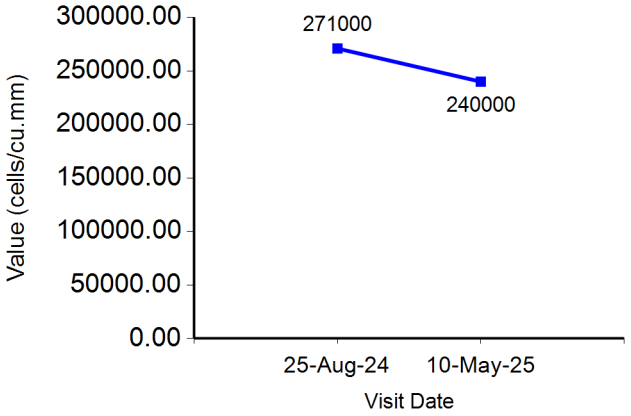
ESR - Erythrocyte Sedimentation Rate



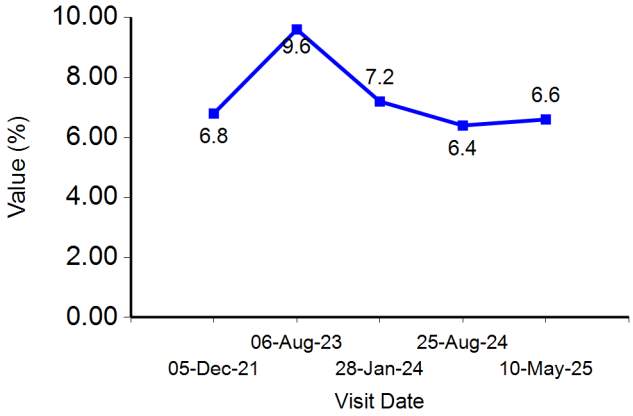
Total Leucocytes (WBC) count



Platelet count



HbA1C- Glycated Haemoglobin



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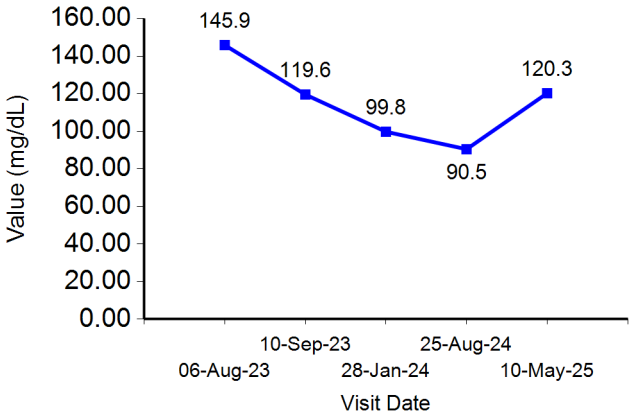
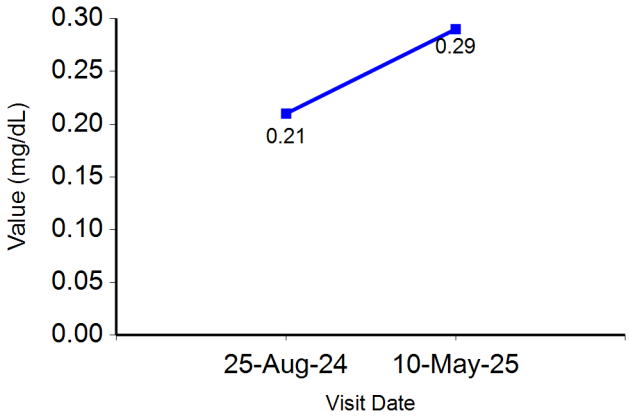
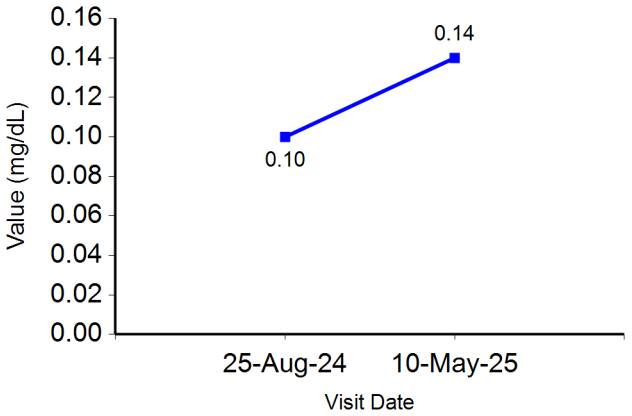
Glucose fasting



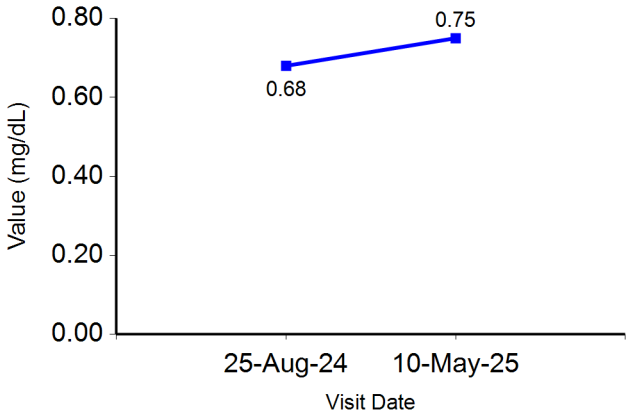
Bilirubin-Total



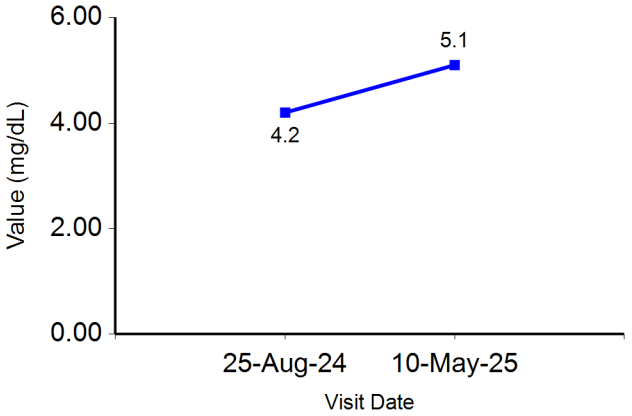
Bilirubin-Direct



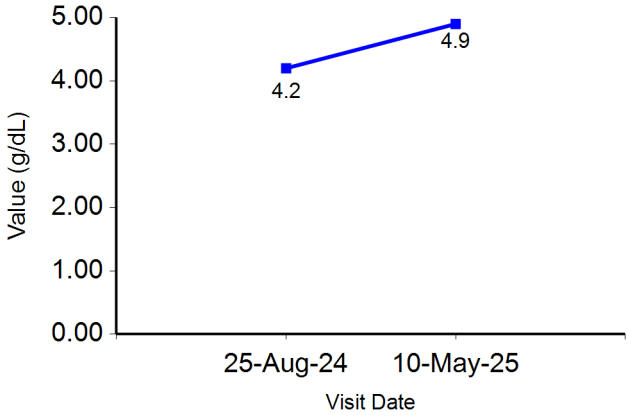
Creatinine



Uric Acid



Albumin



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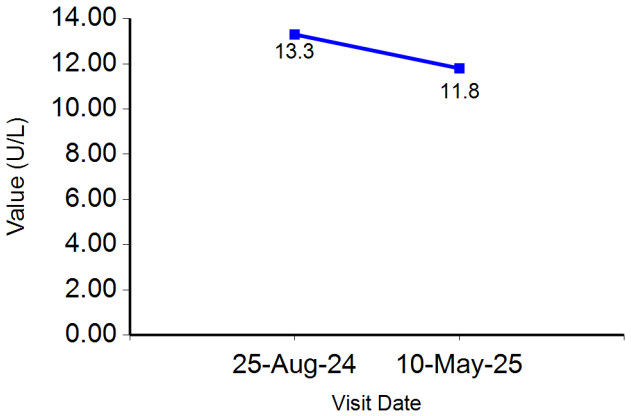
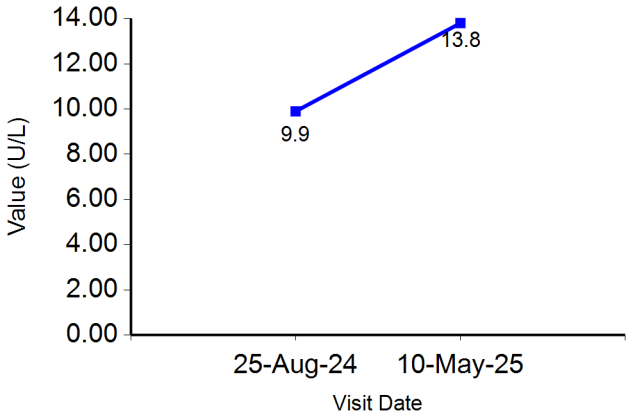
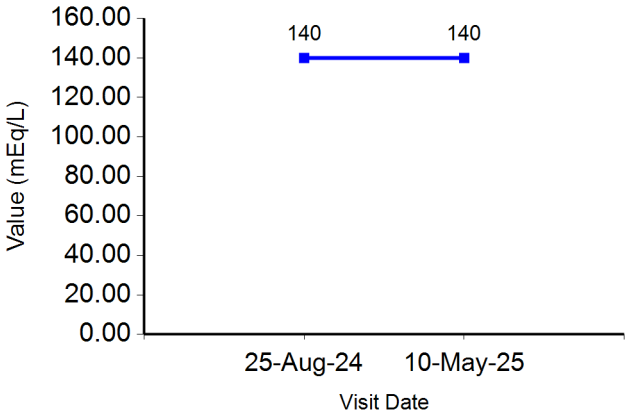
SGOT (AST)



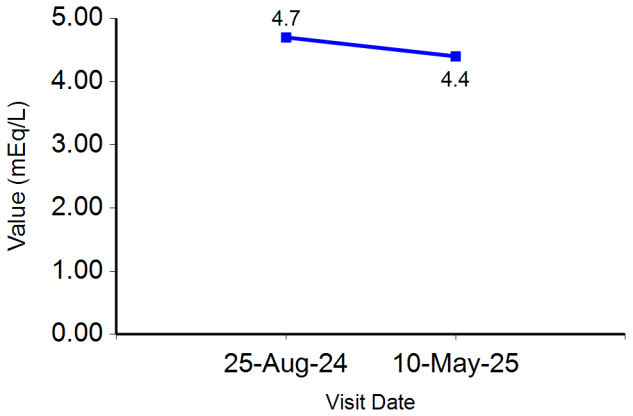
SGPT (ALT)



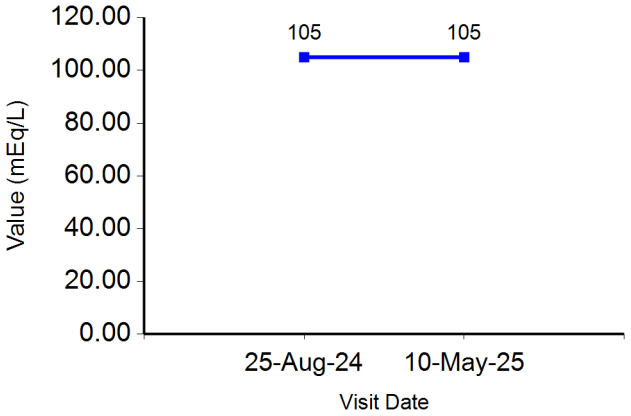
Sodium



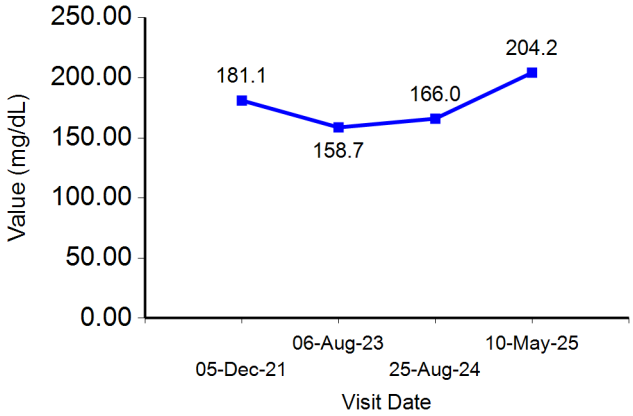
Potassium



Chlorides



Cholesterol-Total



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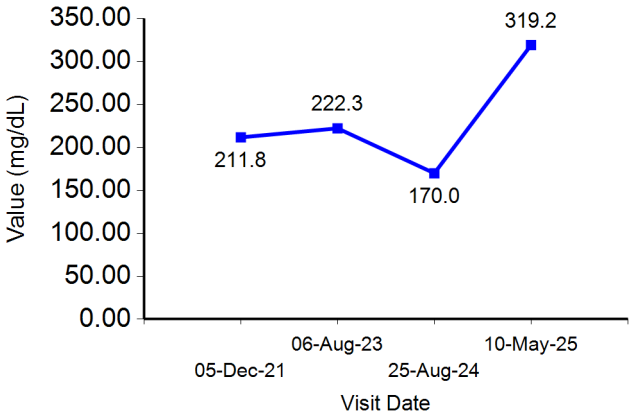
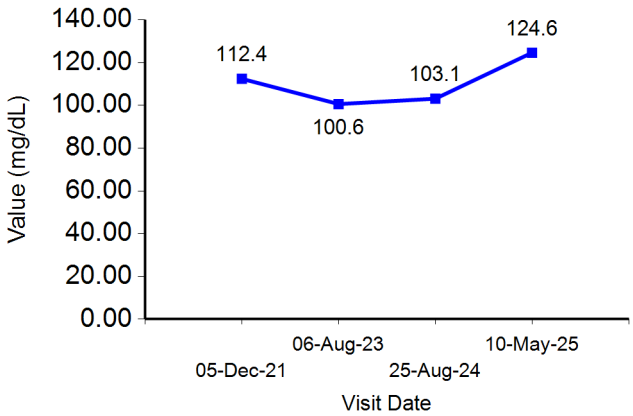
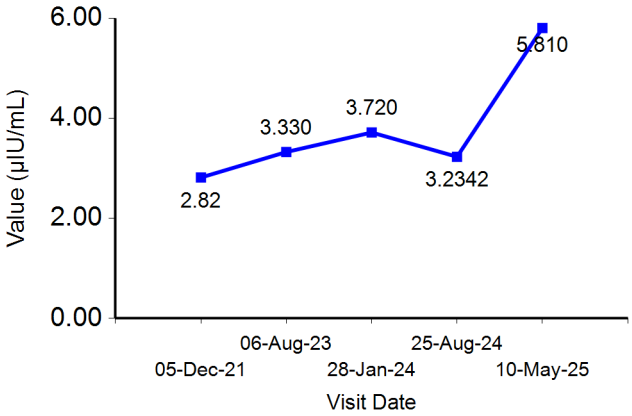
Triglycerides



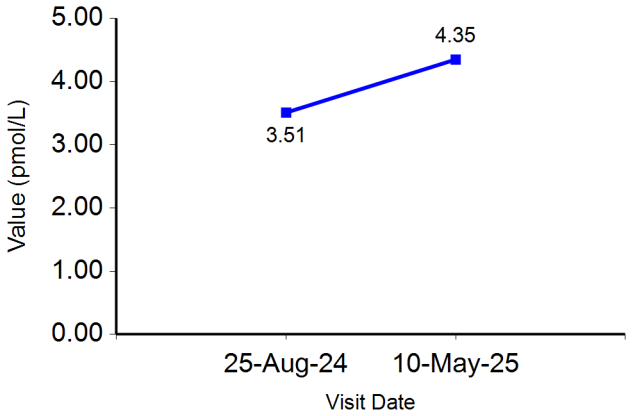
LDL Cholesterol



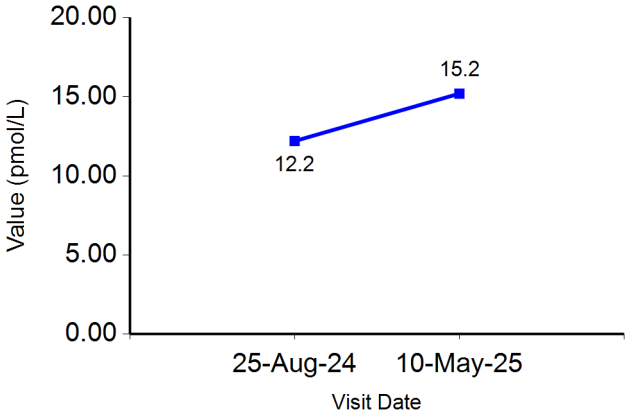
TSH(Ultrasensitive)



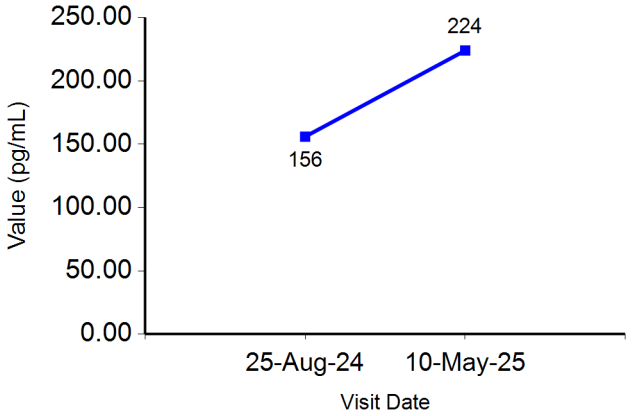
Free T3



Free T4



Vitamin B12 level



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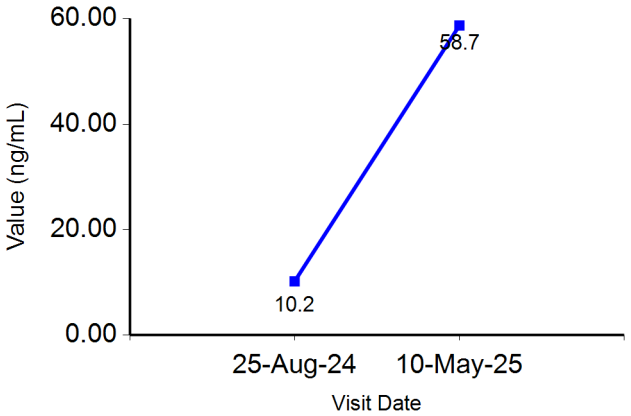
25 Hydroxy (OH) Vit D



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TruHealth Vital
HbA1c- Glycated Haemoglobin
(EDTA Whole Blood)

Investigation	Observed Value	Unit	Biological Reference Interval
HbA1C- Glycated Haemoglobin (High-Performance Liquid Chromatography (HPLC))	**6.6**	%	Normal: 4.8-5.6 Pre - Diabetes: 5.7-6.4 Diabetes: >= 6.4 Recent ADA guidelines 2018 (Please see the recent change of reference range as per guidelines)

Comments :Suggested: Follow up.

Estimated Average Glucose (eAG)	142.7	mg/dL

In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control.

Excellent Control - 6 to 7 %,
Fair to Good Control - 7 to 8 %,
Unsatisfactory Control - 8 to 10 %
and Poor Control - More than 10 % .

NOTE : Any condition altering red cell life will alter the GHB values. Low Hb% values may not correlate with GHB. GHB value should not be taken as a sole criteria for diagnosis. GHB gives average Blood Glucose level for the period of 10 - 12 wks & it need not correlate with blood sugar levels.
For Geriatric group, HbA1c reference range depends upon Co-morbid conditions.

METHOD : HPLC .

MC-5857

Tests marked with NABL symbol are accredited by NABL vide Certificate no MC-5857

Dr. RAVIKUMAR.H.N
MD DCP
KMC NO : 22886

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CBC Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
Erythrocytes			
Haemoglobin (Hb) (Photometric measurement)	14.6	gm%	11.5-16.5
Erythrocyte (RBC) Count (Electric Impedance method)	4.99	mill/cu.mm	3.5-5.5
PCV (Packed Cell Volume) (Calculated)	42.5	%	37-47
MCV (Mean Corpuscular Volume) (Derived from RBC Histogram)	85.1	fL	76-96
MCH (Mean Corpuscular Hb) (Calculated)	29.2	pg	27-32
MCHC (Mean Corpuscular Hb Concn.) (Calculated)	34.3	%	30-35
RDW (Red Cell Distribution Width) (Derived from RBC Histogram)	13.7	%	< 15
Leucocytes			
Total Leucocytes (WBC) Count (Electric Impedance method)	8,500	cells/cu.mm	4000-11500
Absolute Neutrophils Count (Calculated)	5907.5	cells/cu.mm	Upto 8625
Absolute Lymphocyte Count (Calculated)	1683.0	cells/cu.mm	Upto 4600
Absolute Monocyte Count (Calculated)	671.5	cells/cu.mm	Upto 1380
Absolute Eosinophil Count (Calculated)	195.5	cells/cu.mm	Upto 444
Absolute Basophil Count (Calculated)	42.5	cells/cu.mm	Upto 110
Neutrophils (Electric Impedance method)	69.5	%	40-75
Lymphocytes (Electric Impedance method)	**19.8**	%	20-40
Monocytes (Electric Impedance method)	7.9	%	2-12
Eosinophils (Electric Impedance method)	**2.3**	%	3-8
Basophils (Electric Impedance method)	0.5	%	0-2

Dr. Varsha B
MBBS,MD PATHOLOGY,KMC NO:117036

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Investigation	Observed Value	Unit	Biological Reference Interval
Platelets			
Platelet count (Electric Impedance method)	240000	cells/cu.mm	150000-450000
MPV (Mean Platelet Volume)	7.0	fL	

Note:- Kindly note change in reference ranges.
EDTA Whole Blood-Tests done on Automated Five Part Cell Counter. (RBC and Platelet count by impedance/Hydrodynamic focusing,WBC and differential by VCS technology/Impedance/Flow cytometry.Rest are calculated parameters).All Abnormal Haemograms are reviewed confirmed microscopically.Differential count is based on approximately 10,000 cells.

Tests marked with NABL symbol are accredited by NABL vide Certificate no MC-5857

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Investigation	Observed Value	Unit	Biological Reference Interval
Sodium, Serum (Serum,Ion Selective Electrode (ISE))	140	mEq/L	135-150

Interpretation:

- Low levels are noted in prolonged vomiting or diarrhea, diminished reabsorption in the kidney and excessive fluid retention. High levels are seen in case of excessive fluid loss, high salt intake and increased kidney reabsorption.

Potassium, Serum (Serum,Ion Selective Electrode (ISE))	4.4	mEq/L	3.5-5.4

Interpretation:

- Low levels are noted in reduced intake of dietary potassium or excessive loss of potassium from the body due to diarrhea, prolonged vomiting or increased renal excretion. High levels may be caused by dehydration or shock, severe burns, hemolysis, diabetic ketoacidosis, and retention of potassium by the kidney. False low or high values are found if the sample is not collected properly. Eg.Hemolysed samples, Anticoagulated samples.

Chloride, Serum (Serum,Ion Selective Electrode (ISE))	105	mEq/L	95-107

Interpretation:

- Low levels are noted in reduced dietary intake, prolonged vomiting and reduced renal reabsorption as well as some forms of acidosis and alkalosis. High levels are found in dehydration, kidney failure, some forms of acidosis, high dietary or parenteral chloride intake, and salicylate poisoning.

Gamma GT (GGTP) (Serum,Enzymatic)	18	U/L	< 40

Dr. SRILALITHA P
MBBS, MD
KMC:1219 KTK

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Investigation	Observed Value	Unit	Biological Reference Interval
Glucose Fasting (Plasma-F,Hexokinase)	**120.3**	mg/dL	Normal: 70-99 Impaired Fasting Glucose(IFG): 100-125 Diabetes mellitus: >= 126 (on more than one occasion) (American diabetes association guidelines 2022)

Medical Remarks: Suggested: Follow up.

Note: An individual may show higher fasting glucose level in comparison to post prandial glucose level due to following reasons : The glycaemic index and response to food consumed, Changes in body composition, Increased insulin response and sensitivity, Alimentary hypoglycemia, Renal glycosuria, Effect of oral hypoglycaemics & Insulin treatment.

Associated Tests: HbA1c (H0018), Diabetes Profile – Maxi (D0021),HOMA Index (H0275), Insulin (I0275).

Please see Recent Change in Reference Range.

Bilirubin Total, Direct, Indirect

Bilirubin Total (Serum,Diazo method)	0.29	mg/dL	0.1-1.2

In certain normal individuals Total Bilirubin upto 2.0 mg/dL is considered as normal.
Ref: Tietz 5th edition.

Bilirubin Direct (Serum,Diazo method)	0.14	mg/dL	0-0.50
Bilirubin- Indirect (Serum,Calculated)	0.15	mg/dL	0-0.8

Proteins, Serum

Total Protein (Serum,Biuret test)	8.0	gm/dL	6.4-8.3
Albumin, Serum (Serum,Bromocresol Green (BCG))	4.9	gm/dL	3.5-5.2
Globulin (Serum,Calculated)	3.1	gm/dL	2.3-3.5
Albumin/Globulin Ratio (Serum,Calculated)	1.6		1.1-1.8
SGPT (ALT) (Serum,Enzymatic IFCC)	13.8	U/L	< 33
SGOT (AST) (Serum,Enzymatic IFCC)	11.8	U/L	< 32
Creatinine, Serum (Serum,Jaffes method)	0.75	mg/dL	0.5-1.11

Creatinine is a by product of muscle energy metaboism is produced at a constant rate according to the muscle mass of the individual. Creatinine is a fairly reliable indicator of kidney function. Creatinine levels should be correlated with clinical data and

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Ms. SHASHIKALA N
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RAJAJINAGAR ,,Bangalore North Bangalo..
Tel No : +919886259840
PIN No: 560055
PID NO: P1832100080128
Age: 50.5 Year(s) Sex: Female

Reference: DR.MAHESH CHANDRA PAI
Sample Collected At:
RV Metropolis
#76/10, 15th Cross Rd, 4th Main Rd, Malleshwaram,banglore.
Processing Location:- Metropolis Healthcare Ltd. #76/10,15th cross,Malleshwaram,Bangalore.

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used in conjunction with other renal function tests for complete assessment.

BUN, Serum (Serum,Calculated)	9.5	mg/dL	6-20
Uric Acid, Serum (Serum,Enzymatic)	5.1	mg/dL	2.4-5.7

Note : Since Uric Acid is elevated in number disease process where there is increased metabolic turnover,correlation with clinical data is requested,for interpretation of the results.

Phosphorus, Serum (Serum,Molybdate UV)	4.38	mg/dL	2.5-4.5
Calcium, Serum (Serum,NM-BAPTA)	9.88	mg/dL	8.7-10.7

Calcium is a very sensitive test and any haemolysis in the sample may give rise to false high values.False low values are seen if any anticoagulated sample is used.We recommend to repeat the test with fresh sample in case the values are abnormal.

Lipid Profile - 2 (Mini - Fasting)

Cholesterol Total, Serum (Serum,Enzymatic)	**204.2**	mg/dL	Desirable: < 200 Borderline: 200-240 High: > 240
Triglycerides, Serum (Serum,Enzymatic)	**319.2**	mg/dL	Normal: < 150 Borderline High: 150-199 High: 200-499 Very High : >= 500
HDL Cholesterol Direct (Serum,Enzymatic)	46.3	mg/dL	Low: < 40 High: >= 60 Normal: 40-60
Non HDL Cholesterol (Serum,Calculated)	**157.9**	mg/dL	Optimal: < 130 Desirable: 130-159 Borderline high: 160-189 High: 190-220 Very High: >= 220
LDL Cholesterol (Serum,Direct Homogeneous method)	**124.6**	mg/dL	Optimal: < 100 Near Optimal: 100-129 Borderline high: 130-159 High: 160-189 Very High: >= 190
VLDL Cholesterol (Serum,Calculated)	63.8	mg/dL	Upto 40
LDL/HDL Ratio (Serum,Calculated)	2.7		2.5-3.5

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Note: Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

Cholestrol / HDL Ratio (Serum,Calculated)	4.4		< 5.0
Alkaline Phosphatase, Serum (Serum,Enzymatic)	96	U/L	35-104

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Investigation	Observed Value	Unit	Biological Reference Interval
TruHealth Vital			
ESR (Erythrocyte Sedimentation Rate) (EDTA Whole Blood,Optical Electronic Automated method)	5	mm/hr	< 20

Method: Automated Westergren

Interpretation:

1. It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
2. It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
3. It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

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Investigation	Observed Value	Unit	Biological Reference Interval
FT3 (Free Triiodothyronine) (Serum,Electrochemiluminescence immunoassay (ECLIA))	4.35	pmol/L	3.07-6.75
Note : Please note recent change in Reference range			
FT4 (Free Thyroxine) (Serum,Electrochemiluminescence immunoassay (ECLIA))	15.2	pmol/L	11.97-21.88
Note : Please note recent change in Reference range			
TSH (Thyroid Stimulating Hormone) - Ultrasensitive, Serum (Serum,Electrochemiluminescence immunoassay (ECLIA))	**5.810**	µIU/mL	0.54-5.30
Medical Remarks: Repeat TSH (Test code: T0130) at a later date.			

INTERPRETATION :

TSH
- High → Free T4
  - Low → Hypothyroidism
  - Normal → Subclinical Hypothyroidism
- Normal
- Low → Free T4
  - High → Hyperthyroidism
  - Normal → Subclinical Hyperthyroidism

Interpretation :

- Increased TSH is seen with intake of Iodine, Lithium, Amiodarone drugs and also indicates considerable physiologic & seasonal variation.
- Decreased TSH values require correlation with patient age & clinical symptoms and seen with intake of few drugs e.g. L-dopa, glucocorticoids.
- Transient alteration in TSH is seen in non-thyroidal illness like severe infections, liver disease, renal and heart failure, severe burns, trauma and surgery etc.

Clinical Utility: Levels of TSH are used for monitoring of thyroid related disorders.

Caution: Patients on Biotin supplement may have interference in some immunoassays. For sample collection, at least 8-hours wait time is recommended for individuals taking high dose of Biotin (more than 5 mg per day) supplements.

Note: TSH levels may fluctuate based on few factors such as pregnancy, illness and age. Also, time of sample collection, technologies used to analyze the test, usage of certain drugs, diet may have impact on TSH levels. TSH may show around 50% variation even when done at different times of day due to its association with circadian rhythm.

Associated Tests: T3 (T0029), T4 (T0031) free T3 (T0028), free T4 (T0030), reverse T3 (R1004), Thyroid Antibodies (T0061), Thyroid Comprehensive Profile-1 (T0062)

Reference:

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KMC:1219 KTK

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- Clinical Chemistry 50:12, 2338-2344 (2004) and Ind J Clin Biochem (Apr-June 2014) 29(2):189–195.
- Ref: Arch Pathol Lab Med—Vol 141, November 2017.
- Fisher DA. Physiological variations in thyroid hormones: physiological and pathophysiological considerations. Clin Chem. 1996 Jan;42(1):135-9. PMID: 8565215
- Tietz text book of Clinical Chemistry & Molecular Diagnostics 8th Edition.

Note : Please note recent change in Reference range

Vitamin B12 (Cyanocobalamin) (Serum,Electrochemiluminescence immunoassay (ECLIA))	224	pg/mL	197-771

Interpretation :

1. Vit B12 levels are decreased in megaloblastic anemia, partial/total gastrectomy, pernicious anemia, peripheral neuropathies, chronic alcoholism, senile dementia, and treated epilepsy.
2. An associated increase in homocysteine levels is an independent risk marker for cardiovascular disease and deep vein thrombosis.
3. Holo Transcobalamin II levels are a more accurate marker of active VitB12 component.

Caution:

- Patients on Biotin supplement may have interference in some immunoassays. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended.

Disclaimer:

- High levels of Vitamin B12 may be due to exogenous supplementation. Kindly correlate clinically.

Associated Tests

- Active Vitamin B12 (V0012), Homocysteine reflex Vitamin B12-folate serum (H0310), Homocysteine Serum (H0254),RBC Folate R0007.

Reference:

1. Package insert
2. Arch Pathol Lab Med—Vol 141, November 2017

Vitamin D Total - 25 Hydroxy (OH) (Serum,Electrochemiluminescence immunoassay (ECLIA))	58.7	ng/mL	Deficiency: < 10 Insufficiency: 10-30 Sufficiency: 30-100 Toxicity: > 100

Interpretation:

- Vitamin D is a fat soluble vitamin and exists in two main forms as D3 & D2. Both are converted to 25(OH) vitamin D in liver.
- For diagnosis of vitamin D deficiency, it is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum iPTH & serum alkaline phosphatase
- During monitoring of oral vitamin D therapy- suggested testing of serum 25(OH) vitamin D is after 12 weeks or 3 months of treatment.

Caution:

- Patients on Biotin supplement may have interference in some immunoassays. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended.

Disclaimer:

- The required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal (especially winter) & individual variability depending on age, body fat, sun exposure, physical activity, genetic factors (especially variable vitamin D receptor responses), associated liver or renal diseases, malabsorption syndromes and calcium

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or magnesium deficiency.

- Vitamin D toxicity is known but very rare. Kindly correlate clinically, repeat with fresh sample if indicated.

Associated Tests:

- iPTH-Intact Molecule Parathyroid hormone Serum/Plasma (P0114), Calcium(C0017), Vitamin D plus profile(V0016)

Reference:

1. Package insert
2. Arch Pathol Lab Med—Vol 141, November 2017

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Routine Examination Profile - Urine

Investigation	Observed Value	Unit	Biological Reference Interval
TruHealth Vital			
Routine Examination Profile - Urine			
General Examination			
Colour	Pale Yellow		Pale Yellow
Transparency (Appearance)	Clear		Clear
Reaction (pH)	5		4.6-8.0
Specific Gravity	1.017		1.001-1.035
Chemical Examination (Automated Urine Chemistry)			
Urine Protein (Albumin) (Protein Error Principle)	Absent		Absent
Urine Glucose (Glucose Oxidase-Peroxidase (GOD-POD))	**Present 4+ (Re-Checked)**		Absent
Urine Ketones (Acetone) (Legals Test)	Absent		Absent
Urobilinogen (Diazo method)	Normal		Normal
Nitrite (Greiss Reaction)	Negative		Negative
Bile Salt & Pigments	Negative		Negative
Bilirubin (Diazo method)	Negative		Negative
Microscopic Examination Cuvette Based Imaging Tech			
Red blood cells	0.0	/hpf	0-2.3
Pus cells (WBCs)	0.2	/hpf	0-2.7
Epithelial cells	0.9	/hpf	0-1.1
Crystals	Absent	/hpf	0-1.4
Cast	Absent	/hpf	0-0.5
Bacteria	Absent	/hpf	0-29.5

Comments :Bacteria value: 12.7

Yeast cells	Absent		Absent

Note : Nitrite indicates presence of bacterial infection. False Positive and False Negative results are known to occur. Bacteria are

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normally present in urine and correlation with leucocyte count and culture is clinically important. Mild proteinuria may be seen in normal individuals especially during fever, strenuous exercise & dehydration. Repetition is required in all cases of proteinuria. Crystals may be seen in normal individuals also & to be clinically correlated. RBCs may be seen during menstrual flow in females, in urine sample as well.

All urine samples are checked for adequacy and suitability before examination

-- End of Report --

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